- Tight clothing in men may raise the temperature in the scrotum and affect sperm production.
- Under developed ovaries or testes.
- Female may develop antibodies against her partner's sperm.
- Males may develop an autoimmune response to their own sperm.

All women are born with ovaries, but some do not have functional uterus. This condition is called **Mayer-Rokitansky syndrome**.

3.8. Assisted Reproductive Technology (ART)

A collection of procedures, which includes the handling of gametes and/or embryos outside the body to achieve pregnancy is known as **Assisted Reproductive Technology**. It increases the chance of pregnancy in infertile couples. ART includes intra-uterine insemination (IUI) or Artificial insemination (AI), *in vitro* fertilization, (IVF) Embryo transfer (ET), Zygote intra-fallopian transfer (ZIFT), Gamete intrafallopian transfer (GIFT), Intra-cytoplasmic sperm injection (ICSI), Preimplantation genetic diagnosis, oocyte and sperm donation and surrogacy.

Intra-Uterine Insemination (IUI)/ Artificial insemination (AI)

This is a procedure to treat infertile men with low sperm count. The semen is collected either from the husband or from a healthy donor and is introduced into the uterus through the vagina by a catheter after stimulating the ovaries to produce more ova. The sperms swim towards the fallopian tubes to fertilize the egg, resulting in normal pregnancy.

In Vitro Fertilization (IVF) or Test tube baby

In this technique, sperm and eggs are allowed to unite outside the body in a laboratory. One or more fertilized eggs may be transferred into the woman's uterus, where they may implant in the uterine lining and develop. Excess embryos may be cryopreserved (frozen) for future use. Initially, IVF was used to treat women with blocked, damaged, or absent fallopian tubes. Today, IVF is used to treat many causes of infertility. The basic steps in an IVF treatment cycle are ovarian stimulation, egg retrieval, fertilization, embryo culture, and embryo transfer.

Egg retrieval is done by minor surgery under general anesthesia, using ultrasound guide after 34 to 37 hours of hCG (human chorionic gonadotropin) injection. The eggs are prepared and stripped from the surrounding cells. At the same time, sperm preparation is done using a special media. After preparing the sperms, the eggs are brought together. 10,000-1,00,000 motile sperms are needed for each egg. Then the zygote is allowed to divide to form 8 celled blastomere and then transferred into the uterus for a successful pregnancy. The transfer of an embryo with more than 8 blastomeres stage into uterus is called **Embryo transfer technique**.



Cryopreservation (or freezing) of embryos is often used when there are more embryos than needed for a single IVF transfer. Embryo cryopreservation can provide an additional opportunity for pregnancy, through a **Frozen embryo transfer** (FET), without undergoing another ovarian stimulation and retrieval.

Zygote Intra-Fallopian Transfer (ZIFT)

As in IVF, the zygote upto 8 blastomere stage is transferred to the fallopian tube by laparoscopy. The zygote continues its natural divisions and migrates towards the uterus where it gets implanted.

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